



“WE HAVE NEVER SEEN THESE MANY COMPANIES MOVE THIS FAST INTO A SINGLE THERAPEUTIC CATEGORY. THE LEVEL OF COMPETITION WILL COMPRESS PRICES FURTHER”

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“IN INDIA, THE ‘MEDICALISATION’ OF OBESITY OR SEMAGLUTIDE IS STILL RECENT. I HOPE THAT OVER TIME OBESITY TREATMENT GETS COVERAGE”

VIKRANT SHROTRIYA

MANAGING DIRECTOR, NOVO NORDISK INDIA

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remains under medical supervision,” says Shrotriya.

THE RIPPLE EFFECTS

Though much of the attention on semaglutide has been focused on its ability to help shed weight, its clinical case extends well beyond that. Trial data published in *The New England Journal of Medicine*’s SELECT study showed a 20% reduction in major cardiovascular events in overweight and obese adults without diabetes who were treated with semaglutide. Recent evidence from Indian clinics also supports its use in managing non-alcoholic fatty liver disease, a condition that can cause the liver to swell and lead in extreme cases to cirrhosis.

The hospital sector is watching closely. This is because interventions like bariatric surgery to treat

obesity have increased steadily in Indian cities over the past decade. Data from the Obesity and Metabolic Surgery Society of India shows the number of procedures has roughly doubled since 2018, though it remains a niche intervention.

As a medical alternative becomes more affordable, some surgeons expect the risk-benefit calculation for borderline surgical candidates to shift. Others are more confident that surgery will remain relevant for severe obesity cases.

“Bariatric surgery will continue to have a role, especially in severe obesity. While injections are expanding treatment options, they typically need to be taken long term, whereas surgery is a one-time intervention,” says Dr Pradeep Chowbey, Chairman of Max Institute of Laparoscopic, Endoscopic

and Bariatric Surgery. “In many cases, we also use these therapies to prepare patients for surgery, rather than replace it entirely.”

In diagnostics, the impact is more direct. Structured drug-led weight management programmes require testing and regular monitoring. Metropolis has introduced three structured, scientifically curated test packages under its GLP-1 series. “As GLP-1 therapies gain traction across India, they must be supported by a reliable diagnostic framework,” says Surendran Chennakoti, Managing Director of Metropolis Healthcare Ltd.

In the food and nutrition sector, research from Euromonitor International shows that GLP-1 drug adoption in the US has already begun to affect sales in high-calorie snack and beverage categories.

REAL CHALLENGE

India’s generic pharma sector is well placed to supply semaglutide at scale and at an accessible .

The real challenge now lies in converting access into long-term outcomes at scale, experts say. A report by the Public Health Foundation of India published in 2025 identifies physician training, patient awareness and insurance coverage as the three structural constraints.

Data from the Insurance Regulatory and Development Authority of India shows that metabolic and obesity-related therapies are largely excluded from retail health insurance policies. “In India, the ‘medicalisation’ of obesity or semaglutide is still recent. I hope that over time obesity treatment gets coverage,” says Shrotriya.

“The insurance gap is critical. Without coverage, lower prices will not translate into sustained use,” Mitra says. **BT**

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